

Email to GP at completion of post-remission therapy

Subject: Maria Garcia, DOB 22.07.1965 (AML007) — AML (NPM1+) update and ongoing follow-up

Dear Dr. Santos,

I'm writing with a concise update on Ms. Maria Garcia (59 y/o) following treatment for acute myeloid leukemia.

Diagnosis & Biology:

- AML diagnosed 05.11.2021. ELN 2022 **Favorable** risk.
- Molecular: **NPM1 mutation**; no FLT3-ITD/TKD; normal karyotype.
- Presentation: anemia, thrombocytopenia, WBC 18 with 45% blasts; gingival swelling.

Treatment Course:

- **Induction:** "7+3" (cytarabine + daunorubicin) initiated **12.11.2021**, uncomplicated aside from febrile neutropenia responding to piperacillin-tazobactam.
- **Response:** Day-28 marrow CR; MRD by NPM1 qPCR $<10^{-4}$.
- **Consolidation:** High-dose cytarabine (HiDAC) $\times 3$ cycles (Jan–Mar 2022) with expected cytopenias; no neurotoxicity.
- **Maintenance:** We elected **oral azacitidine (CC-486) 300 mg D1–14 q28d** starting May 2022 given age and to reinforce MRD negativity. Completed 12 cycles by April 2023.

Current Status (as of March 2024, ~28 months from induction start):

- Clinically well, ECOG 0–1. Working part-time; mild fatigue only.
- CBCs stable: WBC 4.6 (ANC 2.1), Hgb 12.3 g/dL, Plt $212 \times 10^9/L$.
- CMP unremarkable; creatinine 0.9 mg/dL; AST/ALT <30 .
- NPM1 MRD qPCR **undetectable** on serial assays (latest 01.03.2024).
- No transfusions since 2022; no infections requiring admission after consolidation.

Comorbidities & Meds:

- Hypertension (lisinopril 10 mg qd).
- Type 2 diabetes (metformin 1,000 mg bid; A1c 6.7%).
- Hyperlipidemia (atorvastatin 20 mg qHS).

- Osteoarthritis knees (topical diclofenac PRN).
- Vaccinations updated (influenza, pneumococcal, COVID booster).
- No anticoagulants/antiplatelets.

Late Effects / Counseling:

- Ocular dryness and mild peripheral neuropathy (grade 1) likely post-cytarabine; using artificial tears; gabapentin 100 mg qHS PRN.
- Bone health: calcium/vitamin D; DEXA planned given post-menopausal status and corticosteroid exposures during induction.
- Cancer screening: routine age-appropriate screening recommended; avoid live vaccines; annual dermatology skin check.

Plan:

- Continue **surveillance only**: clinic q3–4 months with CBC; NPM1 qPCR every 4–6 months in year 3, spacing out if negative.
- No ongoing chemotherapy; central line removed.
- Should cytopenias, infections, or NPM1 re-emergence occur, we'll re-stage with marrow and consider re-induction or targeted therapy per evolving standards, with transplant consultation if relapse biology warrants.

Please let me know if you notice new bruising, persistent infections, unintentional weight loss, or abnormal labs between our visits. I appreciate your partnership in Ms. Garcia's care.

Warm regards,
[Name], MD
Hem/Onc Service
Direct line: [redacted]